



MVAIR

MEDVOICE AI RECEPTIONIST

CONFIDENTIAL — INTERNAL STRATEGY DOCUMENT

Market Reality & *Survival* Research Report

An honest, evidence-based assessment of where MVAIR stands today, what the market demands, the gap between the two, and the 90-day path to a first paying clinic.

PREPARED

19 June 2026

SCOPE

US Primary · CA / UK / AU Secondary

PURPOSE

Go-to-Market Decision & Pitch Preparation

VERSION

1.0

Confidence key — **H** High (primary / corroborating sources) ·
M Medium (single credible source) · **L** Low (directional / vendor-sourced)

[CP] Competitor Page

[IR] Industry Report

[AE] Analyst Estimate

[PR] Press / Funding

[VG] Vendor Guide

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★ Verdict

Single most important finding: MVAIR can survive and earn — but it needs a compliance spine (BAA chain + recording proxy + secure auth) before it can legally accept money from a US clinic. Once legally sellable, the one action that most increases odds is winning **one paying, BAA-covered, PMS-integrated clinic** — because integration is simultaneously the moat, the churn antidote, and the proof point that unlocks referrals and any investor conversation.

Market Reality

The pain MVAIR addresses is real and quantified: independent practices miss 20–38% of inbound calls, each missed new-patient call is worth roughly \$125–\$850 depending on specialty, and annual leakage per practice runs from ~\$12K to well over \$100K. But the category has moved from "early" to **contested** — there is a dental-specific, YC-backed incumbent (Arini), an enterprise health-system incumbent (Hyro, \$95M raised), and a long tail of horizontal AI receptionists priced from \$65/mo. MVAIR is entering a market where "AI answers the phone" is already table stakes, not a differentiator.

Market Size

- Practices miss **20–35% of calls**; average missed call ~\$125–\$200, new-patient calls \$300–\$850 lifetime value; a practice missing ~10 new-patient calls/month loses \$100K+/year. (M–L [IR]/[VG] — *directionally consistent; treat exact dollar figures as marketing-grade.*)
- US dental front-desk staffing alone** cited by Arini (YC) at ~\$7B/year inside a ~\$220B dental industry. (M [PR].)
- Top-down "conversational AI in healthcare": ~\$10.8B (2023) → ~\$80.5B by 2032 at ~25% CAGR. (L [AE] — *PR-driven research reports; useful for a pitch slide, not for planning.*)
- Most credible number for a pitch:** per-practice ROI (recovered missed-call revenue vs. subscription cost) — not the inflated top-down TAM.

AI Voice Receptionist Competitors — Medical / Dental-Specific

Company	Focus	Funding	Key Differentiators	Pricing	Conf.
Arini	Dental / DSO	YC W24, ~\$500K seed	Dentrix, Eaglesoft, Open Dental, Denticon, CareStack; insurance verify; recall; emergency triage; 30-day pilot; "100% HIPAA"	Mid-three-figures/mo per location	H [CP] [PR]
Hyro	Enterprise health systems	~\$95M total	45+ health systems, 30M+ patients; Epic integration; SOC 2 Type II; not an SMB competitor	Enterprise / custom	H [PR]
Viva AI	Dental	Early	Outbound recall + payments	~\$399–\$699/mo	M [CP]
AINORA	Dental	Early	Multilingual	Not public	M [CP]
Patientdesk	Dental	Early	Insurance verification	Not public	M [CP]
TensorLinks	Dental	Early	PMS depth (tiered)	\$399–\$699/mo	M [CP]

Horizontal AI Competitors (Cross-Vertical, Include Medical Templates)

- **My AI Front Desk** — multi-vertical, ~\$65–\$200/mo; dental is one of many shallow templates. [CP, M]
- **Retell AI / Synthflow / Bland / Goodcall** — infrastructure + no-code builders. Retell and Synthflow notably bundle **HIPAA + BAA without an enterprise contract**. [CP/VG, H]

Critical infrastructure note: MVAIR is built on Vapi — meaning MVAIR may compete against products built on the same underlying infrastructure. The infrastructure layer is not the moat. Retell AI bundles HIPAA + BAA at ~\$0.07/min with no \$1,000/mo fixed-cost floor; Vapi charges ~\$1,000–\$2,000/mo as a flat add-on. This is the most important economic finding in this report (see Section 5).

Non-AI Incumbents MVAIR Displaces

Incumbent	Pricing	Failure Modes
Human answering service	\$500–\$3,000/mo medical-grade	Passive message-taking, callback delay lets leads go cold, one call at a time, after-hours gaps
In-house receptionist	\$42K–\$65K/yr fully loaded	8×5 coverage only, turnover, sick days, multitasking → missed calls during check-in
Voicemail / nothing	"Free"	62% of callers hang up on voicemail; ~41% of call attempts happen after hours
Offshore virtual assistants	Lower-cost human	Human latency, scheduling/timezone limits, one-at-a-time

Pricing M–H [CP]; failure-mode framing M [VG].

Earliest-Adopter Specialties (US, Ranked)

1. **Dental** — clearest ROI math, standardized PMS landscape (Dentrix / Open Dental / Eaglesoft), recall culture. Also where funded competition already is.
2. **Allied health / physio / chiro** — high call volume, simpler scheduling, Jane App common in CA/AU/UK.
3. **Dermatology / aesthetics / med-spa** — cash-pay, marketing-driven, conversion-sensitive.
4. **Mental health** — high demand, but sensitivity and triage risk raise the compliance and liability bar.
5. **GP / primary care** — high volume but insurance complexity, EHR fragmentation, triage liability make it harder.

Confidence: M–L [CP]/[VG] — competitor vertical focus and dental-specific funding concentration as proxy.

US Regulatory Environment — The Hard Gate

Handling patient name + phone + reason-for-visit on a clinic's behalf makes MVAIR a HIPAA Business Associate. Selling to a US practice legally requires:

1. A signed **BAA between MVAIR and each clinic**
2. Flow-down BAAs with every subcontractor that touches PHI: Vapi, the LLM provider, TTS provider, Twilio, and the hosting provider

OCR has issued **six-figure penalties for missing BAAs alone — before any breach occurred.** The absence of the agreement is itself the violation. (H [IR] — HHS OCR / Omnibus Rule §164.504(e).)

Also required: encryption in transit + at rest, audit logging, RBAC, breach-notification procedures, US data residency. CA CMIA and some two-party call-recording-consent states add a further layer.

Buyer Psychology & Decision Process

For a single independent practice the buyer is the owner-clinician or practice/office manager, and the decision is faster and more emotional than enterprise health-IT sales suggest. The dominant emotion is **trust and fear of error** ("what if your AI books the wrong thing or mishandles an emergency?"), not price. The fastest path to a yes is a live demo call they can hear, a low-risk parallel-running pilot, and a peer reference.

Who Decides (by Clinic Size)

Clinic Size	Decision Maker	Sales Pattern
Solo / 1–3 providers	Owner-dentist/physician (office manager as gatekeeper)	Short chain; can sign in weeks
Small group (4–10)	Practice manager evaluates; owner signs	2-stakeholder; 4–8 week cycle
Multi-location / DSO	Operations / IT + procurement	Enterprise pattern: long cycle, many stakeholders, RFP likely

What Triggers a Search

- Missed-call pain becoming visible (patient complaints, empty schedule slots)
- **Front-desk turnover** — the most common acute trigger
- Opening a second location or a marketing push that outpaces inbound capacity
- After-hours voicemail losing new-patient leads overnight

Confidence: M [VG].

Objections

- **"Will patients hate talking to a robot?"** — trust in patient experience is the #1 emotional objection
- **"Is this HIPAA compliant, will you sign a BAA?"** — a hard yes/no gate; no BAA = no deal
- **AI error risk** — wrong bookings, double-bookings, mishandled emergencies, hallucinated answers
- **Loss of control** — "what happens to my existing phone number and my PMS?"

Proof Required Before Signing

- A **live demo call** they can place themselves and hear the AI in action
- A **30-day trial/pilot** running alongside existing staff before full cutover
- A **peer reference** from a comparable practice in the same specialty
- Visible **audit logs / call transcripts** reviewable at any time from the dashboard

This is exactly the proof structure Arini productizes. Confidence: H.

Sales Cycle Length

- **Enterprise medical software:** ~12 months, ~9 decision-makers (M [VG])
- **SMB single-clinic (target):** 2–8 weeks from demo to pilot start, then 30 days of monitored pilot before full commitment. Plan around this number, not the enterprise figure.

Pricing Model Preference

- Practices prefer **flat monthly per location** — per-minute billing creates budget anxiety and discourages routing calls to the AI (defeating the purpose)
- **Psychologically safe price points:** solo practice \$199–\$499/mo; group practice \$499–\$999/mo per location; setup fees \$0–\$1,500 are normal
- **All-in market reality:** \$400–\$900/mo single location, despite \$199 homepage anchors (M [CP])

Phone Number Portability — Non-Negotiable

Clinics **will not re-print signage or re-train patients on a new number**. They must keep their existing number. Support **call forwarding** from their carrier (easy, days to set up) and **number porting** (harder, paperwork-heavy) as an option. Treat "keep your number" as a hard requirement, not a differentiating feature. Confidence: H.

Gap Analysis: MVAIR Today vs. Market Minimum Viable

MVAIR has a working contract-first vertical slice (call → book → log → dashboard, validated on real calls), which is genuinely ahead of a slide-deck competitor. But against the **market minimum to charge a real US clinic**, three gaps are existential: it cannot legally handle PHI (no BAA chain), it has no PMS/EHR integration (the only durable moat and #1 retention driver), and it only books — it can't reschedule or cancel. Everything else is High or Medium and closeable in days to weeks.

Capability	MVAIR Today	Market Minimum	Gap Severity	Est. Effort to Close
HIPAA — BAA Chain	No BAA with clinics; no subcontractor BAAs; Vapi HIPAA add-on not enabled	Signed BAA w/ clinic + flow-down BAAs (Vapi, LLM, TTS, Twilio, host)	Critical	2–4 wks + legal review + Vapi HIPAA spend or migrate to Retell
PMS / EHR Integration	Google Calendar only	At least one real PMS (Open Dental most accessible; Jane App is approval-only)	Critical (moat + reschedule)	2–8 wks per integration
Reschedule / Cancel	Book only — no reschedule or cancel flows	Book + reschedule + cancel (table stakes)	High	1–2 wks
Encryption / Audit / RBAC	JWT in localStorage (XSS-exposed); SQLite at rest unencrypted; cross-tenant guards exist	Encryption at rest + transit, audit logs, RBAC, httpOnly cookies	High	1–2 wks
Recording Privacy	Audio served from public storage.vapi.ai URLs — accessible to anyone with link	Proxied, expiring signed URLs; access-logged	High	3–5 days (already scoped)
Phone Number Portability	Demo CTAs only; unclear carrier support	Call forwarding from existing number (min); porting (ideal)	High	Days (forwarding) / Weeks (porting)
Multilingual — Spanish	English only	Spanish at minimum in US market	High	Days (Vapi supports; needs prompt + flow work)
SMS Confirmation	Lead capture; phone in calendar event only	SMS booking confirmation now an expectation	High	3–5 days (Twilio SMS)
Uptime SLA / Failover	Manual start; ngrok dependency; single server; no process management	99.9% target, monitored, no single point of failure	High	1–3 wks (ngrok → real domain, PM2, monitoring)
Billing / Subscriptions	None — no ability to charge	Stripe subscriptions, per-location plans, dunning	High	1 wk
After-Hours & Holiday Flows	Business hours enforced in tool + system prompt	Holiday calendars, after-hours triage / voicemail fallback	Medium	3–5 days
Multi-Location Within Tenant	Multi-tenant (clinic-level) only — not multi-location within a tenant	Shared brand, multiple sites, per-site routing/calendars	Medium	1–2 wks

Capability	MVAIR Today	Market Minimum	Gap Severity	Est. Effort to Close
Self-Onboarding	JSON config editor (developer-grade — not usable by a clinic admin)	Non-technical clinic admin can configure without developer help	Medium	2–4 wks (defer for now)
Support / Escalation Path	None defined — no documented support or emergency escalation	Documented support channel + emergency escalation to clinic staff	Medium	Days (process, not code)

Critical rows = cannot legally or functionally sell without. **High** rows = days-to-weeks to close. **Medium** rows = close after clinic #1 is live. The single-server SQLite architecture is NOT a near-term risk (see Section 5).

Go-To-Market Reality

A bootstrapped solo founder does not out-market a YC company — you **out-focus** it. Win one specialty + one PMS you can actually integrate, sell founder-to-owner with a live demo and a 30-day parallel-running pilot, and earn referrals. Freemium would actively damage positioning in a trust-sensitive regulated market.

First-Client Acquisition (Bootstrapped Reality, Ranked)

1. **Founder-led direct outreach to a warm niche** — one specialty, ideally a personal connection or local proximity. Arini's founders literally went door-to-door to dental practices.
2. **Referral** — one delighted clinic referring peers is the dominant SMB-healthcare acquisition channel; build for it from day one by making the pilot experience remarkable.
3. **Channel partners (medium-term)**: PMS vendors with partner programs, dental/medical billing companies, practice-management consultants, healthcare incubators.
4. **Conferences** — useful for credibility later; expensive and slow at this stage.

Pilot Structure That De-Risks the Clinic

30-day parallel pilot: AI runs after-hours only first, fully monitored, clinic reviews every transcript, gradual cutover to 100% of calls once trust is established. No long-term contract during pilot. This is now the *expected* trial structure — Arini and others have trained the market to expect it. Confidence: H [CP].

Freemium Decision

Freemium: No. In a regulated, trust-sensitive category a free tier signals "not serious about your data" and attracts non-buyers. A **free 30-day pilot** (high-touch, time-boxed, BAA-covered) is the correct risk-reducer. An open-ended free tier destroys positioning and attracts tyre-kickers who will never convert. Confidence: M.

Certification / Endorsement

Formal medical-association endorsement is slow and not worth chasing pre-revenue. What *does* accelerate sales: **SOC 2 Type I, a clean BAA, a Trust Center page, and named peer references from comparable practices.** Confidence: M.

Channel Partners Worth Targeting

Partner Type	Why	Note
Open Dental	Accessible API, integration ecosystem, dental-native	Best first PMS integration target
Dental / medical billing companies	Already trusted by clinics — natural upsell	White-label or referral arrangement
Practice consultants / brokers	Influence purchase decisions across many clinics	Commission-based referral — low overhead
DSOs / MSOs	One deal = many locations, lower per-location CAC	Requires multi-location + PMS integration first
Jane App	Dominant in allied health (CA / AU / UK)	API is approval-only / currently gated — verify access before targeting allied health
Healthcare incubators	Warm intro to vetted early-adopter clinics	Useful for credibility + deal flow

Revenue & Scaling Model

The decisive finding of this report is in the unit economics: **building on Vapi saddles MVAIR with a ~\$1,000–\$2,000/month HIPAA fixed cost that makes the first 1–5 clinics structurally unprofitable**. Retell AI (and Synthflow) include HIPAA + BAA without that fixed floor. Re-evaluating the voice provider may matter more to survival than any feature on the roadmap.

MRR Needed to Fund a 2-Person Team for 18 Months

Lean assumptions: 2 modest founder salaries + infrastructure + tooling ≈ \$12K–\$20K/mo burn. Break-even target: **\$15K–\$20K MRR**. At common price points:

Monthly Price / Clinic	Clinics Needed to Break Even
\$299 / mo	~50–67 clinics
\$499 / mo	~30–40 clinics
\$799 / mo	~19–25 clinics

Note: a deferred-salary solo founder needs far less. Confidence: M.

The Vapi HIPAA Cost Problem (Critical)

- Vapi all-in variable cost: ~**\$0.13–\$0.33/min** (platform + STT + LLM + TTS + telephony). A typical clinic's call volume costs only ~**\$30–\$100/mo variable**.
- Vapi HIPAA / BAA flat add-on: ~**\$1,000–\$2,000/mo** (H [CP], multiple sources).
- Retell AI alternative: ~**\$0.07/min with HIPAA + BAA included**, no enterprise plan, no fixed-cost floor.

Unit Economics at \$499/mo Plan — Vapi HIPAA Add-On vs. Retell

Clinics on Platform	HIPAA Fixed Cost / Clinic	Variable Cost / Clinic	Gross Margin / Clinic @ \$499 (Vapi)
1 clinic	~\$1,000	~\$65	–\$566 (deeply negative)
5 clinics	~\$200	~\$65	+\$234 (thin)
10 clinics	~\$100	~\$65	+\$334 (workable)
25+ clinics	~\$40	~\$65	+\$394 (healthy)

Resolution options — choose before signing clinic #1:

1. **Migrate voice layer to Retell AI** — eliminates the \$1,000/mo floor; HIPAA + BAA bundled; variable cost drops from ~\$0.13–\$0.33 to ~\$0.07/min. Highest impact per hour spent.
2. Absorb the Vapi HIPAA cost as customer-acquisition expense for the first cohort and re-evaluate at 10 clinics.
3. Price first cohort at \$799+/mo (founding-clinic annual rate) to compress the loss window.

Natural Expansion Revenue Path

Solo clinic → multi-location → group practice → **DSO/MSO** (one contract, many locations, lower per-location CAC, higher retention). Expansion revenue (add-ons: recall outreach, SMS confirmations, analytics reporting) is where vertical SaaS makes its real margin. Confidence: H.

Architecture at Scale

Clinic Count	Architecture Verdict	Action
1–50	Single-server SQLite is fine — better-sqlite3 is fast for this read/write profile	No action needed
50–200	Write-concurrency + single-point-of-failure limits appear	Migrate to managed Postgres; stateless app instances
500+	Multi-region, read replicas, observability stack	A problem worth having — plan path, execute when needed

Architecture is **not** a near-term survival risk. Resist premature scaling work.

Churn Drivers in SMB Vertical SaaS

- SMB SaaS churns **~3–7%/month (≈30–58%/yr)**; **~70% of churn happens in the first 90 days** — onboarding quality is the highest-leverage retention lever. (H [IR].)
- **The antidote is PMS integration** — embeds in core operations and makes the product too critical to rip out. Solves moat and retention simultaneously.
- Additional mitigants: annual contracts (30–40% lower churn than monthly), fast time-to-first-value (<7 days), visible ROI reporting the manager can show the owner.

90-Day Survival Plan

The next 90 days should be spent making MVAIR **legally and operationally sellable to one real clinic** — not adding features. The single most important build is the compliance + PHI-handling spine. The single most important non-build is getting one paying, integrated clinic live and generating visible ROI.

Days 1–30 — Become Legally Sellable

- Priority 1** **Voice provider economics decision:** Vapi HIPAA add-on (~\$1,000–\$2,000/mo flat) vs. migrating to Retell AI (~\$0.07/min, HIPAA + BAA bundled, no fixed floor). This decision gates everything else — the unit model, the BAA chain, and the first invoice.
- Priority 2** **Recording proxy** — serve audio through `/api/recordings/:id` with expiring signed URLs. Closes the public Vapi URL PHI exposure. Already scoped; 3–5 days of work.
- Priority 3** **Auth hardening** — JWT → httpOnly cookies; encryption at rest on SQLite; role-based access control; audit log table. Required for any HIPAA claim.
- Priority 4** **Reschedule + Cancel flows** — table-stakes product gap. No clinic will commit if patients can't cancel or move an appointment via the same AI that booked it.
- Priority 5** **Production deployment** — Railway + Vercel, replace ngrok with a real stable domain, PM2 or equivalent for process management. One real URL to share with a prospect.

Minimum Legal Step Before Charging Money

A signed **BAA between MVAIR and the clinic** + executed BAAs with every PHI-touching subcontractor (voice provider, LLM, TTS, Twilio, host). Have your own BAA template **reviewed by a healthcare attorney** (a few hundred to low thousands of dollars — non-optional).

Taking money from a clinic that routes patient data through MVAIR, without this in place, is a standalone HIPAA violation exposed to OCR penalties even with zero breaches.

What to Defer (Do Not Gold-Plate)

- Visual (non-JSON) config editor — JSON editor works; ship it after first paying client
- Multi-location within a single tenant — needed for DSO; not needed for clinic #1
- Analytics dashboards beyond sessions / leads / appointments panels
- Webhook pingback for on-call provider notification on emergency
- Additional languages beyond Spanish
- Second PMS integration — build it only after clinic #1 proves the model
- Architecture migration (SQLite → Postgres) — not needed until 30–40 clinics
- ElevenLabs voice upgrade — Vapi's built-in voice is sufficient for the demo

First Sales Motion (Exact)

Target: One specialty + one PMS you can integrate today. Recommended: **dental on Open Dental** (accessible API, clear ROI math, standardized scheduling) — accepting that Arini is there — *or* a less-contested allied-health niche where you have a warm intro (noting Jane App's API is currently gated, so plan Google Calendar-based booking there initially).

Who to call: Owner-clinicians and practice managers of 1–3-provider independents. Local or warm-referred first.

What to say: *"You're losing \$X/month to missed and after-hours calls. I'll run my AI receptionist alongside your front desk for 30 days, free, after-hours first. You review every transcript. If it's not booking real appointments cleanly, you owe nothing."*

What to offer: Free 30-day monitored parallel pilot → convert to a founding-clinic annual rate (\$499–\$799/mo) with a signed BAA at cutover.

Day-by-Day Success Milestones

Day 30	Deployed, compliance spine live, BAA template attorney-reviewed, demo line a prospect can call from their phone, recording proxy + reschedule/cancel shipped. One pilot clinic verbally committed to starting.
Day 60	One clinic running a live, monitored pilot (after-hours first), real patient calls booking cleanly, transcripts reviewed weekly with the clinic manager, zero compliance incidents. First PMS integration in progress or complete.
Day 90	First paying clinic on a signed BAA + annual plan, running ≥ 1 week at full call volume, with a documented repeatable provisioning + compliance playbook and one written reference quote for use in outreach.

Verdict

MVAIR can survive and earn — but not as currently positioned, and not yet. The working, contract-validated vertical slice (call → book → log → dashboard, proven on real phone calls) is a genuine asset that puts it ahead of vaporware competitors. The underlying market pain — missed-call revenue leakage — is real and quantifiable at the per-clinic level.

However, today the product is a thinner version of what a funded, dental-native incumbent already ships, in a market where "AI answers the phone" is no longer a differentiator, sitting on an infrastructure choice (Vapi) whose **~\$1,000–\$2,000/month HIPAA fixed cost makes the first handful of clinics structurally unprofitable**. The moat is not voice quality and not price — it is **compliance depth plus PMS/EHR integration**, and MVAIR has neither in place yet.

The single thing that would most increase MVAIR's odds in the next 60 days: get one real, paying, BAA-covered clinic live and integrated with its practice-management system. PMS integration is simultaneously the only durable moat, the dominant defense against the 70%-in-90-days SMB churn curve, and the proof point that unlocks referrals, partners, and any investor conversation. Resolve the voice-provider HIPAA economics first — it gates the unit model — then go win clinic #1.

Confidence and source-type tags are embedded throughout. Market-size figures from vendor blogs and PR-driven research reports should be treated as directional only.

Competitor pricing and HIPAA/BAA requirements are the highest-confidence findings in this report.

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